

the Harvard School of Public Health this year. In India, it shows, 20 to 40 per cent more battered women get addicted to tobacco.

"The biggest contributor to women's smoking has been the promotion of the cigarette as a cultural icon of sophistication, glamour and sexual allure," holds Madhu Kishwar, editor of the women's magazine, *Manushi*, who piloted anti-smoking campaigns when the first brand of cigarettes aimed at upwardly mobile women, Ms, was launched in 1989 in the country. "The ads showed strong, happy women in western clothes and affluent settings. Tobacco companies push cigarettes down women's throats in the name of modernity," she scoffs. Professionally independent women still form clandestine market targets for tobacco companies. "These are women of a higher socio-economic status, who aspire for a western lifestyle. Such as, those who work for the multinationals, media or those who are studying in schools of mass communication and international organisations," reports Mira Aghi, a behavioural scientist who consults with the UN and is the force behind Advocacy Forum for Tobacco Control.

Cigarette comes across as a glamour symbol in Bollywood, too. Whereas earlier films portrayed either vamps or village women smoking, contemporary films profile middle-class women (WHO, 2003, Bollywood: Victim or Ally). As per a 2005 study (Report on Tobacco Control in India) sponsored by the Health Ministry on tobacco use in Indian cinema, 26 per cent Hindi films were found to have a heroine who smoked; in 72 per cent films, smoking stood for a glamorous lifestyle, independence, free-spiritedness (remember Vimmi of *Bunty Aur Babli?*). To the editor of the report, cardiologist Dr K. Srinath Reddy, who had won the WHO award for Outstanding Contribution to Global Tobacco Control in 2003, "Cigarette companies have developed sophisticated campaigns, many of which circumvent the Indian tobacco advertising ban." The only way out of the smokescreen is to understand their marketing strategies and plug the loopholes in India's tobacco control legislation.

But the legal trail is strewn with loopholes. First, anti-tobacco legislations in India lack teeth. Take the 1975 statutory warning ("Cigarette smoking is injurious to health") on packets. For the last 33 years they have not really worked. The prohibition on selling tobacco to underage consumers in 1998 fell through due to lack of enforcement. A series of Acts between 2000 and 2004 banning smoking in public places, tobacco sponsorship and all advertising except at point of purchase, has made the tobacco lobby's marketing strategies more sophisticated. In 2006, the proposed ban on smoking onscreen remains unresolved. New plans of getting skulls and crossbones on cigarette packets are in the offing this year. Will they work?

"The main problem is that health is both a federal as well



HOW SMOKING HARMS WOMEN MORE

- International research shows, women stand to lose about eight years of productive life to smoking
- They are 25 times more likely to develop cancer
- Heart attacks may occur 16-19 years earlier than in non-smokers
- Chances of heart attack are 20 times more for those using oral contraceptives
- Smoking can mess up women's reproductive systems, fertility and trigger mental disorders
- Smoking during pregnancy is known to cause still or premature births, as well as spontaneous abortions
- Passive smokers breathe side-stream smoke, which contains three times more nicotine and tar and 50 times more cancer-causing substances

as state subject. The Centre may pass laws, but those will have to be implemented at the level of the states," rues Thyagarajan. "There have been problems with implementation, both cash and capacity being in short supply." With the 11th Plan allocating Rs 450 crore for anti-tobacco projects, the ministry is now gearing up on capacity building. But just as cigarette commercials did not address women directly, the government's intervention plans also do not seem to target women specifically. Yet smoking among women is not a simple habit. It's governed by social and psychological factors. "The cessation strategies need to be different from those used for men," Aghi points out. "Using health as a reason to quit smoking is too limited. Only a positive approach, taking into account the subtleties of women's lives, can work."

Once upon a time, Amrita Pritam wrote of "smoking away the woes". Caught between contradictory images of who they are and what they want to be, women seem to be doing just that. Women are resisting patriarchal stigmas, yet asserting themselves in a manner borrowed from men. But unless the opposition to smoking drops its moral tone, it is doubtful how far they will manage to flick the ash of despondency from the flame of personal choice. ■